

Paper 1: Medicine Through Time with the Western Front

"How did the horrific conditions of trench warfare drive rapid medical innovation?"

MASTERY PACK: WESTERN FRONT

Operative / Student Name: _____

Target: Total Recall of 150 Crucial Questions

RESTRICTED ACCESS - MASTER YOUR MEMORY

How to Hack Your Memory

(Rules of Engagement)

1. Never Just Read

Reading the answers feels like learning, but it's an illusion. You must force your brain to struggle to find the answer. The struggle is what builds memory pathways.

2. Cover, Recall, Check

Cover the answer sheet. Write your answer down in the notes section (or say it out loud). Only then are you allowed to check 'The Vault'.

3. Track Your Threat Level

Use the R-A-G (Red, Amber, Green) checkboxes next to every question after you attempt it.

- ☐ **Green:** I nailed it instantly.
- ☐ **Amber:** I got it, but I had to think hard.
- ☐ **Red:** My mind went blank. (Revisit these tomorrow).

4. The Interrogation (Homework Protocol)

True mastery requires testing under pressure. Hand 'The Vault' (answers) to your parent or guardian. They will test you on 20 random questions. They hold the score; they hold the signature.

KT5.1: What was the historical and medical context of the Western Front?

Q#	The Target Question	R	A	G	Notes / My Answer
1	By 1900, how had surgical practice in civilian hospitals evolved from Joseph Lister's earlier methods?	☐	☐	☐	
2	Which of the following was a key piece of technology used in aseptic surgery by the outbreak of the First World War?	☐	☐	☐	
3	In what year did the German physicist Wilhelm Roentgen accidentally discover X-rays?	☐	☐	☐	
4	Which of the following was a major limitation of using early X-ray machines in 1914?	☐	☐	☐	
5	Why were early X-ray machines highly unsuitable for the fast-paced, mobile environment of a battlefield in 1914?	☐	☐	☐	
6	In 1901, the Austrian doctor Karl Landsteiner made a breakthrough that revolutionized blood transfusions. What did he discover?	☐	☐	☐	
7	Before the discovery of sodium citrate in 1915, what was the massive problem with performing blood transfusions on the battlefield?	☐	☐	☐	
8	When the British Expeditionary Force (BEF) arrived on the Western Front in 1914, what geographical feature made the medical situation catastrophic?	☐	☐	☐	
9	What is the military definition of a 'salient', such as the famous one at Ypres?	☐	☐	☐	
10	Which major British offensive, launched in 1916, became a medical disaster with over	☐	☐	☐	

Q#	The Target Question	R	A	G	Notes / My Answer
	57,000 casualties on the very first day?				
11	What unique medical facility was established during the Battle of Arras in 1917?	☐	☐	☐	
12	During the Battle of Cambrai in 1917, what groundbreaking medical logistical system was used for the first time by Oswald Robertson?	☐	☐	☐	
13	Why did aseptic surgery (sterile operating theatres) prove to be nearly useless in frontline dressing stations in 1914?	☐	☐	☐	
14	What made transporting wounded soldiers away from the frontline so incredibly difficult?	☐	☐	☐	
15	To what extent was the Royal Army Medical Corps (RAMC) prepared for the medical challenges of the Western Front in 1914?	☐	☐	☐	
16	What did the term 'war of attrition' mean for the soldiers living in the trench system?	☐	☐	☐	
17	Why were horse-drawn ambulances often ineffective in the worst conditions of the Western Front?	☐	☐	☐	
18	Which of the following best describes the layout of the trench system on the Western Front?	☐	☐	☐	
19	In what year did Karl Landsteiner discover that blood group O was the universal donor group?	☐	☐	☐	
20	Which statement accurately summarises the 'historical context' of medicine at the outbreak of war in 1914?	☐	☐	☐	

MISSION DEBRIEF

Score: _____ / 20

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Date: _____

KT5.1: What was the historical and medical context of the Western Front?

Q#	The Target Question	R	A	G	Notes / My Answer
21	What lifestyle choice was definitively linked to lung cancer in the 1950s?	☐	☐	☐	
22	Name one modern technology used to diagnose lung cancer.	☐	☐	☐	
23	State one way the modern government tries to prevent people from smoking.	☐	☐	☐	
24	When did the First World War begin and end?	☐	☐	☐	
25	What was the 'Western Front'?	☐	☐	☐	
26	Name the three major battles fought by the British on the Western Front.	☐	☐	☐	
27	What disease did Edward Jenner vaccine against in 1796?	☐	☐	☐	
28	What was established in Britain in 1948 to provide free medical care?	☐	☐	☐	
29	Why did the terrain of Ypres make evacuation and medical treatment very difficult?	☐	☐	☐	
30	What did the battle of Cambrai (1917) demonstrate about tanks?	☐	☐	☐	
31	What was the direct environmental cause of 'Trench Foot' on the Western Front?	☐	☐	☐	
32	What were the symptoms of severe, untreated Trench Foot?	☐	☐	☐	
33	How did the British Army attempt to clinically prevent the spread of Trench Foot in permanently flooded trenches?	☐	☐	☐	
34	What was 'Trench Fever', and what were its primary	☐	☐	☐	

Q#	The Target Question	R	A	G	Notes / My Answer
	symptoms?				
35	It took years for medical officers to discover the cause of Trench Fever. What was eventually found to be the transmitter?	☐	☐	☐	
36	How did the British Army strategically adapt to combat the epidemic of Trench Fever once its cause was discovered?	☐	☐	☐	
37	What was the 'Ypres Salient'?	☐	☐	☐	
38	Why was the First Battle of Ypres (Autumn 1914) strategically critical for the British?	☐	☐	☐	
39	What terrifying new weapon was first used by the German Army at the Second Battle of Ypres in 1915?	☐	☐	☐	
40	During the fight for Hill 60 in April 1915, what tactic did the British successfully use to defeat the German positions?	☐	☐	☐	

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KT5.2: How did the trench environment create new medical challenges?

Q#	The Target Question	R	A	G	Notes / My Answer
41	Why did the British launch the massive Battle of the Somme in 1916?	☐	☐	☐	
42	What made the Battle of the Somme a 'medical nightmare' on its very first day?	☐	☐	☐	
43	Because the ground at Arras was chalky and easy to tunnel through, what incredible feat did British and New Zealand miners achieve in 1917?	☐	☐	☐	
44	In the organisation of the trench system, what was the primary purpose of the 'support trench', located roughly 80 metres behind the frontline?	☐	☐	☐	
45	Why were trenches deliberately dug in a zig-zag pattern instead of a straight line?	☐	☐	☐	
46	How did the complex, zig-zag design of the trenches unintentionally create severe problems for the medical services?	☐	☐	☐	
47	Why did the army have to rely heavily on horse-drawn ambulance wagons instead of modern motor ambulances near the frontline?	☐	☐	☐	
48	What was the main medical danger caused by delays in evacuating wounded soldiers across the devastated, muddy terrain?	☐	☐	☐	
49	What was 'Dysentery' and how was it spread on the Western Front?	☐	☐	☐	
50	What does the medical condition 'Shell Shock' (now understood	☐	☐	☐	

Q#	The Target Question	R	A	G	Notes / My Answer
	as PTSD) reveal about the environment of the Western Front?				
51	When did the First World War begin and end?	☐	☐	☐	
52	What was the 'Western Front'?	☐	☐	☐	
53	Name the three major battles fought by the British on the Western Front.	☐	☐	☐	
54	What was 'No Man's Land'?	☐	☐	☐	
55	Name the pattern in which trenches were dug.	☐	☐	☐	
56	What was the purpose of the 'duckboards' in the trenches?	☐	☐	☐	
57	Who discovered the cause of cholera in 1854?	☐	☐	☐	
58	Which anesthetic was discovered by James Simpson in 1847?	☐	☐	☐	
59	Explain why the zig-zag pattern of trenches was crucial for survival.	☐	☐	☐	
60	What were the three main parallel lines of trenches?	☐	☐	☐	

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KT5.3: What new illnesses and wounds did soldiers face?

Q#	The Target Question	R	A	G	Notes / My Answer
61	Which type of weapon was responsible for the highest percentage of wounds (roughly 58%) on the Western Front?	☐	☐	☐	
62	Why were deep shrapnel wounds almost guaranteed to become severely infected within hours?	☐	☐	☐	
63	What was biologically unique about the mud in Flanders and France that made it so deadly when dragged into a wound?	☐	☐	☐	
64	What is the defining, fatal characteristic of the 'Gas Gangrene' infection?	☐	☐	☐	
65	Why did traditional 19th-century surgical antiseptics (like carbolic acid spray) fail to cure Gas Gangrene on the Western Front?	☐	☐	☐	
66	How did the British Army successfully reduce the threat of Tetanus infections on the Western Front?	☐	☐	☐	
67	At the start of the war in 1914, why were head injuries disproportionately high (accounting for roughly 20% of wounds)?	☐	☐	☐	
68	What crucial piece of protective equipment was introduced in 1915, drastically reducing fatal head injuries by 80%?	☐	☐	☐	
69	What terrifying new weapon did the German army introduce at the Second Battle of Ypres in April 1915?	☐	☐	☐	
70	What was the physical effect of a Chlorine Gas attack on a soldier?	☐	☐	☐	

Q#	The Target Question	R	A	G	Notes / My Answer
71	Before official gas masks were widely issued in July 1915, how did soldiers desperately attempt to survive gas attacks?	☐	☐	☐	
72	Which poisonous gas, introduced later in 1915, was significantly faster-acting than chlorine and capable of killing an exposed person within two days?	☐	☐	☐	
73	What made Mustard Gas (introduced in 1917) such a horrific and distinct weapon?	☐	☐	☐	
74	Despite the immense psychological terror it caused, why did poison gas actually account for under 5% of all British deaths (around 6,000 soldiers)?	☐	☐	☐	
75	Which debilitating environmental illness was spread by the millions of body lice living in the seams of the soldiers' unwashed uniforms?	☐	☐	☐	
76	How did the British Army attempt to medically prevent the spread of Trench Foot in the flooded trenches?	☐	☐	☐	
77	What was 'Shellshock'?	☐	☐	☐	
78	Which specialist medical facility in Edinburgh was set up specifically to treat British officers suffering from severe Shellshock?	☐	☐	☐	
79	What exact acronym did the British Army officially use in medical records to categorise cases of Shellshock?	☐	☐	☐	
80	Why did the brutal environment of the trenches force the Royal Army Medical Corps (RAMC) to dedicate huge resources to 'preventative measures'?	☐	☐	☐	

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KT5.3: What new illnesses and wounds did soldiers face?

Q#	The Target Question	R	A	G	Notes / My Answer
81	What was 'No Man's Land'?	☐	☐	☐	
82	Name the pattern in which trenches were dug.	☐	☐	☐	
83	What was the purpose of the 'duckboards' in the trenches?	☐	☐	☐	
84	What is 'Trench Foot'?	☐	☐	☐	
85	Which weapon caused the most casualties on the Western Front?	☐	☐	☐	
86	Why were wounds on the Western Front highly prone to severe infection?	☐	☐	☐	
87	Who discovered Penicillin by accident in 1928?	☐	☐	☐	
88	What structure did Watson and Crick discover in 1953?	☐	☐	☐	
89	What preventative measure was introduced to stop Trench Foot?	☐	☐	☐	
90	What was 'Shell Shock'?	☐	☐	☐	
91	What was the fundamental purpose of the 'Chain of Evacuation' on the Western Front?	☐	☐	☐	
92	Who had the dangerous task of initially rescuing wounded men directly from No Man's Land under enemy fire?	☐	☐	☐	
93	Where was the Regimental Aid Post (RAP) physically located?	☐	☐	☐	
94	What was the medical capability of the Regimental Medical Officer stationed at the RAP?	☐	☐	☐	
95	Who staffed the Advanced Dressing Stations (ADS) and Main Dressing Stations (MDS)?	☐	☐	☐	

Q#	The Target Question	R	A	G	Notes / My Answer
96	How did the British military respond to the violent, injury-worsening journeys caused by horse-drawn ambulance wagons?	☐	☐	☐	
97	Why did the RAMC still have to rely heavily on horse-drawn wagons (sometimes needing six horses) even after acquiring motor ambulances?	☐	☐	☐	
98	How were massive numbers of stabilized casualties (up to 800 at a time) transported from the CCS to the Base Hospitals?	☐	☐	☐	
99	Where were the Casualty Clearing Stations (CCS) strategically located?	☐	☐	☐	
100	What vital medical sorting process took place at the Casualty Clearing Station (CCS)?	☐	☐	☐	

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KT5.4: How did the RAMC and FANY operate the chain of evacuation?

Q#	The Target Question	R	A	G	Notes / My Answer
101	Because deadly gas gangrene developed so rapidly, how did the role of the Casualty Clearing Station (CCS) critically change during the war?	☐	☐	☐	
102	Where were Base Hospitals located on the Western Front?	☐	☐	☐	
103	As the war progressed and the CCS took over emergency amputations, what did Base Hospitals begin to specialize in?	☐	☐	☐	
104	To cope with the unprecedented scale of casualties, how drastically did the RAMC (Royal Army Medical Corps) expand between 1914 and 1918?	☐	☐	☐	
105	Who were the QAIMNS, whose numbers grew from 300 to 10,000 during the war?	☐	☐	☐	
106	What does the acronym FANY stand for?	☐	☐	☐	
107	What was the most famous and physically demanding frontline role undertaken by the female volunteers of the FANY?	☐	☐	☐	
108	Aside from driving ambulances, what other vital morale and hygiene services did the FANY provide?	☐	☐	☐	
109	What was incredibly unique about 'Thompson's Cave', built by British and Commonwealth troops in 1916?	☐	☐	☐	
110	Why was the underground hospital at Arras highly significant for treating casualties?	☐	☐	☐	

Q#	The Target Question	R	A	G	Notes / My Answer
111	What is 'Trench Foot'?	☐	☐	☐	
112	Which weapon caused the most casualties on the Western Front?	☐	☐	☐	
113	Why were wounds on the Western Front highly prone to severe infection?	☐	☐	☐	
114	What does RAMC stand for?	☐	☐	☐	
115	What was the role of the FANY?	☐	☐	☐	
116	Describe the 'Chain of Evacuation' on the Western Front.	☐	☐	☐	
117	What lifestyle choice was definitively linked to lung cancer in the 1950s?	☐	☐	☐	
118	What was established in Britain in 1948 to provide free medical care?	☐	☐	☐	
119	What was the primary role of the Casualty Clearing Station (CCS)?	☐	☐	☐	
120	Where were the Base Hospitals located and what did they do?	☐	☐	☐	

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KT5.5: What incredible medical advances were forged on the Western Front?

Q#	The Target Question	R	A	G	Notes / My Answer
121	Why was traditional 19th-century aseptic surgery effectively impossible to maintain on the Western Front?	☐	☐	☐	
122	Faced with rampant gas gangrene, surgeons began using a technique called 'Debridement' (Wound Excision). What did this involve?	☐	☐	☐	
123	What was the 'Carrel-Dakin method' used to treat deep infections?	☐	☐	☐	
124	What was the main logistical difficulty with the highly effective Carrel-Dakin solution?	☐	☐	☐	
125	If antiseptics, debridement, and the Carrel-Dakin method all failed to stop the rapid spread of gas gangrene, what was a surgeon's only remaining option to save the patient's life?	☐	☐	☐	
126	Before 1915, what was the estimated mortality (death) rate for a soldier who suffered a fractured femur (thigh bone) from a gunshot or shrapnel wound?	☐	☐	☐	
127	Why did a fractured femur cause such a high death rate during early transport?	☐	☐	☐	
128	Who originally designed the Thomas Splint before the outbreak of the First World War?	☐	☐	☐	
129	How did the Thomas Splint (introduced to the Western Front by Robert Jones in December 1915) miraculously increase the survival rate for thigh fractures from 20% to 82%?	☐	☐	☐	

Q#	The Target Question	R	A	G	Notes / My Answer
130	What diagnostic technology was absolutely essential for accurately locating deeply embedded shrapnel so surgeons could remove it before infection set in?	☐	☐	☐	
131	How was X-Ray technology adapted to deal with the vast number of casualties closer to the frontline?	☐	☐	☐	
132	What was a major technical limitation of the early mobile X-ray units used on the Western Front?	☐	☐	☐	
133	Early blood transfusions (such as those pioneered by Dr Lawrence Bruce Robertson) were heavily limited because blood could not be stored. How did this force doctors to perform transfusions?	☐	☐	☐	
134	In 1915, what chemical did Richard Lewisohn discover could be added to blood to successfully stop it from clotting?	☐	☐	☐	
135	Following Lewisohn's discovery, Richard Weil discovered that blood mixed with sodium citrate could be refrigerated for up to two days. What monumentally significant breakthrough did Francis Rous and James Turner make in 1916?	☐	☐	☐	
136	Which American doctor utilized these chemical discoveries to establish the world's first 'blood depot' (blood bank) before the Battle of Cambrai in 1917?	☐	☐	☐	
137	How did Oswald Hope Robertson successfully store the 22 units of pre-donated universal Type O blood for his pioneering blood bank at Cambrai?	☐	☐	☐	

Q#	The Target Question	R	A	G	Notes / My Answer
138	Which pioneering surgeon developed early plastic surgery techniques to rebuild the severe, disfiguring facial injuries caused by shrapnel?	☐	☐	☐	
139	Why did the brutal environment and massive casualty numbers of the Western Front force rapid medical advancements?	☐	☐	☐	
140	Which of the following represents a major diagnostic limitation of the mobile X-ray units?	☐	☐	☐	

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KT5.5: What incredible medical advances were forged on the Western Front?

Q#	The Target Question	R	A	G	Notes / My Answer
141	What does RAMC stand for?	☐	☐	☐	
142	What was the role of the FANY?	☐	☐	☐	
143	Describe the 'Chain of Evacuation' on the Western Front.	☐	☐	☐	
144	What was the Thomas Splint?	☐	☐	☐	
145	What chemical was discovered in 1915 to prevent blood from clotting?	☐	☐	☐	
146	What did the discovery of blood storage allow surgeons to do?	☐	☐	☐	
147	Who discovered the structure of DNA in 1953?	☐	☐	☐	
148	Who introduced antiseptic surgery using carbolic acid in 1865?	☐	☐	☐	
149	How did WWI affect the use of X-rays in medicine?	☐	☐	☐	
150	What was the 'Carrel-Dakin' method?	☐	☐	☐	

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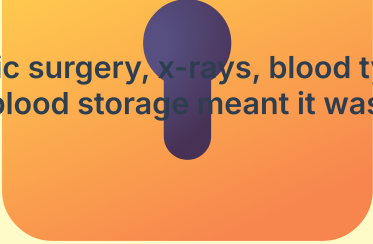
Date: _____

THE VAULT

Restricted Access: Answer Keys

KT5.1: What was the historical and medical context of the Western Front?

1. Surgeons moved from 'antiseptic' surgery (killing germs already in the wound) to 'aseptic' surgery (preventing germs from entering the operating theatre in the first place).
2. The steam autoclave (invented in 1881) used to sterilise surgical instruments.
3. 1895
4. The radiation doses were 1,500 times stronger than today, causing severe burns and hair loss.
5. The glass tubes were incredibly fragile, the machines were too heavy to move easily, and taking a single image could take up to 90 minutes.
6. He discovered three blood groups (A, B, and O), meaning doctors could finally match donor and patient blood types to prevent fatal rejection.
7. Blood coagulated (clotted) as soon as it left the body, meaning it could not be stored in a bank. Transfusions had to be done with the donor and patient connected directly by a tube.
8. The flat agricultural land had its drainage systems destroyed by constant artillery, turning the battlefield into a horrific, waterlogged, muddy swamp.
9. A piece of land that juts out into enemy territory, making it highly dangerous because it is vulnerable to attack from three sides.
10. The Battle of the Somme
11. An extensive underground hospital built into the chalk tunnel network underneath the city, sheltering the wounded from artillery fire.
12. The first ever blood bank, using stored blood to treat soldiers suffering from severe shock.
13. The soldiers' wounds were already deeply infected with anaerobic bacteria from the heavily manured, muddy soil the moment they were injured in the trenches.
14. The deep mud, craters, and destroyed roads made motorized transport impossible near the front, forcing stretcher-bearers to manually carry the wounded for miles.
15. They were completely unprepared for the unique environmental illnesses, the massive casualty numbers, and the horrific shrapnel wounds caused by high-explosive artillery.
16. A static, defensive conflict where millions of men lived for months in unhygienic dugouts, exposed to weather, rats, and human waste while trying to wear down the enemy.
17. The deep mud was so thick that wagons had to be pulled by six horses instead of two, and the severe shaking often worsened the patients' injuries.
18. A complex network of zigzag dugouts consisting of a front line, support trench, reserve trench, and communication trenches linking them.

- 
19. 1907
 20. High-tech medicine (aseptic surgery, x-rays, blood typing) had emerged, but key limitations in mobility and blood storage meant it was not yet adapted for a muddy, static battlefield.
 21. Smoking tobacco.
 22. CT scans or Bronchoscopy.
 23. Banning cigarette advertising, raising taxes, and packaging warning labels.
 24. 1914 to 1918.
 25. A 400-mile line of trenches stretching through Belgium and France.
 26. Ypres, Somme, Cambrai.
 27. Smallpox.
 28. The National Health Service (NHS).
 29. The muddy, low-lying ground became waterlogged and destroyed by artillery shells.
 30. Tanks could break through barbed wire and trenches, though they easily broke down.
 31. Standing for days in cold, waterlogged mud without changing socks.
 32. The feet would swell, blister, turn gangrenous, and often require surgical amputation to save the soldier's life.
 33. By ordering soldiers to change their socks twice a day and systematically rub their feet with whale oil.
 34. A flu-like illness characterized by severe joint pain, shivering, and high fevers.
 35. The millions of body lice living in the seams of the soldiers' unwashed uniforms.
 36. They set up massive delousing stations and bathhouses to disinfect clothing with hot steam.
 37. A vulnerable 'bulge' in the Allied line surrounded by the enemy on three sides, allowing the Germans to fire down from higher ground.
 38. They held onto the salient to stop the Germans advancing to the sea, successfully retaining control of the vital English Channel ports.
 39. Chlorine gas
 40. Offensive mining to tunnel under and blow up the German positions from below.

KT5.2: How did the trench environment create new medical challenges?

41. To relieve extreme military pressure on the French army fighting at Verdun.
42. The British suffered nearly 60,000 casualties in a single day, completely overwhelming the medical services.
43. They dug a 2.5-mile network of tunnels sheltering 25,000 troops, which included a fully functioning underground hospital.
44. It was where troops would retreat to if the frontline came under heavy attack or was overrun.
45. To prevent enemy fire or the blast wave from an exploding artillery shell from travelling straight down the line.
46. It made it incredibly difficult and physically exhausting for stretcher-bearers to safely manoeuvre seriously wounded men around the tight corners while under fire.
47. The heavy motor ambulances frequently got permanently stuck in the deep, waterlogged mud.
48. The delay caused men to die from blood loss or shock, and allowed deep infections like gas gangrene to take hold before they could reach a Casualty Clearing Station.
49. A stomach infection causing severe diarrhea and dehydration, spread by drinking contaminated water from shell holes.
50. That the relentless psychological trauma, noise, and sheer terror of the artillery war caused severe mental breakdowns, resulting in tiredness, nightmares, and uncontrollable shaking.
51. 1914 to 1918.
52. A 400-mile line of trenches stretching through Belgium and France.
53. Ypres, Somme, Cambrai.
54. The dangerous, open ground between the Allied and German front-line trenches.
55. A zig-zag pattern.
56. To keep soldiers' feet out of the mud and standing water at the bottom of the trench.
57. John Snow.
58. Chloroform.
59. It contained the blast of any shell and prevented enemies from shooting straight down the trench.
60. Front-line, Support, and Communication trenches.

KT5.3: What new illnesses and wounds did soldiers face?

61. High-explosive artillery shells and shrapnel.
62. The jagged shrapnel tore through flesh, physically dragging pieces of muddy, bacteria-soaked uniform deep into the wound.
63. It was agricultural farmland that had been heavily fertilized with animal manure, packing the soil with the anaerobic bacteria that caused tetanus and gas gangrene.

64. It was a rapid infection that produced a foul-smelling gas inside dying muscle tissue, turning the flesh black and potentially killing a man within a single day.
65. Antiseptics only cleaned the surface of a wound; they could not reach the lethal bacteria that had been driven deep into the muscle by the force of an explosion.
66. By administering routine anti-tetanus injections to wounded soldiers from late 1914 onwards.
67. Soldiers were fighting in trenches; their bodies were protected by earth, but their heads were exposed while wearing only soft cloth caps.
68. The steel Brodie helmet
69. Chlorine Gas
70. It destroyed the respiratory system, causing victims to slowly suffocate as their lungs filled with fluid.
71. They urinated on cotton pads or handkerchiefs and pressed them to their faces to chemically neutralize the chlorine.
72. Phosgene Gas
73. It was completely odourless and worked slowly, causing severe internal and external blisters that could burn skin straight through uniforms.
74. The rapid development and distribution of highly effective gas masks successfully protected the vast majority of troops.
75. Trench Fever
76. By ordering soldiers to change their socks twice a day and systematically rub their feet with whale oil.
77. A poorly understood psychological trauma (PTSD) caused by the constant bombardment of war, resulting in nightmares, loss of speech, and uncontrollable shaking.
78. Craiglockhart Hospital
79. NYD.N (Not Yet Diagnosed, Nervous)
80. Because the harsh conditions caused so many massive casualties from illnesses (like trench foot and fever) that they had to actively prevent disease just to keep the army fit enough to fight.

KT5.3: What new illnesses and wounds did soldiers face?

81. The dangerous, open ground between the Allied and German front-line trenches.
82. A zig-zag pattern.
83. To keep soldiers' feet out of the mud and standing water at the bottom of the trench.
84. A painful condition caused by prolonged exposure to cold, wet, and muddy conditions, leading to gangrene.
85. Artillery shells and shrapnel.
86. The battlefield soil was heavily fertilized with manure, containing gas gangrene spores.
87. Alexander Fleming.
88. The double helix structure of DNA.
89. Soldiers rubbed whale oil on their feet and changed into dry socks twice a day.
90. A psychological condition caused by the trauma of constant shelling, now known as PTSD.
91. To provide a highly organised, multi-stage system of medical triage to rescue, transport, and treat massive numbers of casualties without overwhelming frontline units.
92. Teams of four to six stretcher-bearers, who carried the casualties by hand through the mud.
93. Within 200m of the frontline, often in a communication trench or dugout.
94. They could only provide immediate first aid (like bandages and tourniquets) to get the walking wounded back to the fight or prepare serious cases for transport; they could not perform surgery.
95. Units of the Field Ambulance, consisting of medical officers, orderlies, and eventually nurses.
96. The Times newspaper launched a public appeal in 1914, raising money to purchase 512 new motor ambulances.
97. Because heavy motor vehicles frequently got bogged down and entirely stuck in the deep, waterlogged mud of the battlefield.
98. On specially designed ambulance trains and canal barges.
99. 7 to 12 miles back from the frontline, safely out of artillery range but positioned near railway lines for rapid transport.
100. Triage, where patients were sorted into three groups: the walking wounded, those needing hospital treatment, and those with no chance of recovery.

KT5.4: How did the RAMC and FANY operate the chain of evacuation?

101. It took over the role of performing critical, life-saving surgery (like amputations) from the Base Hospitals to stop the infection before transport.
102. Near the French and Belgian coastal ports, ready to ship patients back to 'Blighty' (Britain).

103. The longer-term recovery of patients and the development of specialist wards for complex issues like head wounds or gas poisoning.
104. From 9,000 men to 113,000 men.
105. Queen Alexandra's Imperial Military Nursing Service, a corps of highly trained professional military nurses.
106. First Aid Nursing Yeomanry.
107. They drove the motor ambulances, navigating terrible roads and deep mud under active shellfire to transport the wounded.
108. They transported supplies, set up cinemas, and operated mobile bath units that could bathe up to 40 men an hour.
109. It was a massive, fully functioning underground hospital built into existing chalk tunnels beneath the town of Arras.
110. Because it functioned as a fully equipped facility incredibly close to the frontline (with 700 stretcher beds, electricity, and water), while remaining completely safe from enemy artillery.
111. A painful condition caused by prolonged exposure to cold, wet, and muddy conditions, leading to gangrene.
112. Artillery shells and shrapnel.
113. The battlefield soil was heavily fertilized with manure, containing gas gangrene spores.
114. Royal Army Medical Corps.
115. Driving ambulances, moving supplies, and running soup kitchens.
116. A series of medical posts starting at the front line and ending at base hospitals.
117. Smoking tobacco.
118. The National Health Service (NHS).
119. To perform urgent surgery to save life or limbs close to the front line.
120. On the French coast, treating long-term casualties or sending them to Britain.

KT5.5: What incredible medical advances were forged on the Western Front?

- 121. Because the chaotic, muddy casualty stations were impossible to keep perfectly sterile, and traditional carbolic acid did not kill gas gangrene bacteria effectively.
- 122. Aggressively and rapidly cutting away all dead, damaged, and infected tissue from around a wound to physically remove the gangrene before stitching it up.
- 123. A system where tubes pumped a chemical sterilized salt solution directly into deep wounds to continually wash out and kill the infection-causing bacteria.
- 124. The solution only stayed fresh for six hours and had to be constantly made as it was needed.
- 125. Immediate amputation of the infected limb (resulting in over 240,000 lost limbs by 1918).
- 126. 80%
- 127. Because traditional splints did not keep the leg straight, meaning the jagged broken bones ground together, severing major arteries and causing fatal internal bleeding and shock.
- 128. Hugh Owen Thomas
- 129. It rigidly pulled the leg lengthways (traction), completely stopping the broken bones from grinding on each other and preventing fatal blood loss and shock during transport.
- 130. X-Rays
- 131. Six mobile X-ray units (vans) were developed to operate in the British sector, allowing them to be driven directly to Casualty Clearing Stations.
- 132. The delicate glass tubes overheated very quickly, meaning the machines could only be used for an hour before needing to cool down.
- 133. Transfusions had to be done 'arm-to-arm', directly transferring blood from a live donor to the patient before it clotted.
- 134. Sodium Citrate
- 135. They added citrate glucose to the blood which allowed it to be safely stored for up to four weeks.
- 136. Oswald Hope Robertson
- 137. He stored the blood in glass bottles packed with ice and sawdust.
- 138. Harold Gillies
- 139. Because traditional peacetime surgical methods completely failed to treat the complex, infected shrapnel wounds, forcing doctors to innovate radical new techniques out of sheer desperation.
- 140. They could not detect pieces of dirty clothing that had been driven into wounds by shrapnel, which often caused lethal infections.
- 141. Royal Army Medical Corps.
- 142. Driving ambulances, moving supplies, and running soup kitchens.
- 143. A series of medical posts starting at the front line and ending at base hospitals.
- 144. A leg splint that kept the broken femur rigid, reducing blood loss and saving lives.
- 145. Sodium citrate.

- 146. Perform blood transfusions at Casualty Clearing Stations (CCS) using blood banks.
- 147. James Watson and Francis Crick.
- 148. Joseph Lister.
- 149. Mobile X-ray units were deployed close to the front line to locate shrapnel inside wounds.
- 150. Sterilizing deep wounds by constantly flushing them with an antiseptic solution.

Mastery Tracker

Track your total recall score across all 150 questions.

Attempt	Date	Score (/150)	Parent/Guardian Signature	Target for Next Time
1				
2				
3				

Operative Reflection

My ultimate strength in this unit is...

The three specific facts I need to hunt down and memorize tonight are...